



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : تفاحه حسان حسين ابراهيم

العنوان : //

الحي / المدينة : الاميريه

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 06-10-2026

رقم البطاقة : 27208312800341 رقم التليفون : 01015458461

العمر (تاريخ الميلاد) : 31-08-1972 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/ د/ د/ د/

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: تفاحه حسان حسين ابراهيم National ID: 27208312800341	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

تفاحه
حسن حسين ابراهيم
١٣ ش احمد طه مدينة الردوس
الاميرية - القاهرة

٢٧٧٠٨٣١٢٨٠٠٣٤١

JA6588682

٢٠٢١/٠٩ ٢٧٧٠٨٣١٢٨٠٠٣٤١

ربة منزل

أنثى
مسلمة
مطلقة
سيدى شمس الدين حيدرا

البطاقة سارية حتى ٢٠٢٨/٠٩/١١

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION AND DECISION SUPPORT



الهيئة العامة للتأمين الصحي
الإدارة المركزية لتكنولوجيا المعلومات
ودعم القرار

جهة الاصدار : فرع القاهرة

تاريخ الاصدار : 2024-1-3

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 " القوى العاملة والمعاشات والارامل " و كذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسي " وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .
وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية
وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي
والبحث تبين أن

السيد (ة) تفاحه حسان حسين ابراهيم بطاقة رقم 27208312800341

وطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الفئات غير المنتفعة بالتأمين الصحي صدر هذا الخطاب دون اى مسئولية على الهيئة العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الطباعة : 2024-1-3

الموظف المختص
سماح محمود

إفادة مركز نظم المعلومات
يعتمد،،،



مسلسل 1212967

الخطاب يبقي لمدة ثلاثة أشهر من تاريخ الإصدار الا في حالة تغيير حاله المواطن من قبل الهيئة القومية للتأمينات الاجتماعية او وزارة التضامن الاجتماعي او وزارة التربية والتعليم

الإدارة المركزية لتكنولوجيا المعلومات و دعم القرار

Name: تفاحه حسان حسين ابراهيم			
MR. : 10747			
Detailed Medical Report			
		السيد	
تحيه طيبة وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
نتشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient has a history of breast cancer diagnosed in 2023 and has undergone neoadjuvant chemotherapy with 4 cycles of AC and 4 cycles of Taxanes plus anti-HER2 therapy, followed by surgical resection and post-operative radiotherapy. Adjuvant anti-HER2 therapy was completed and the patient has been receiving Femara (letrozole) since 2024 as a maintenance endocrine therapy in the post-menopausal setting. A CT scan of the chest with contrast performed in July 2026 shows a clear operative bed after right mastectomy, prominent left (contralateral) axillary lymph nodes, and several bilateral pulmonary nodules with the largest measuring 1.2 × 1 cm in the left lower anterior segment, likely metastatic. The scan also notes a fatty liver with a 3.5 mm calcific focus suggestive of a granuloma and a bulky left suprarenal gland. The plan is to obtain a breast ultrasound and consider a biopsy of the suspicious lesions, followed by multidisciplinary team discussion based on the results, while continuing Femara.			
تحريراً في ,			
التاريخ :		20-09-2026	
الوقت :			
الطبيب المعالج :		Mariam Adel Hafez	
يعتمد ,			



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : نفاحة حسن ابراهيم

Sex:

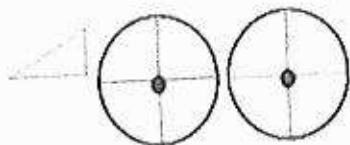
MDT form
3- Medical number:
Female

83134428
Old Med :

Old Med :
1144-2023

2-ID number: 27208312800341

Patient Age : 51 Y



Family history

Family History
-VE

Phone

Phone
01100676690

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status
MARRIED

NRS Score

NRS Score

GROUP

GROUP
C

Date of MDT

Date of MDT
2023-11-19

Co morbidity

Co Morbidity
DM

Menopausal status

Menopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE
Advanced

Clinical data

Breast Imaging :

Mets Workup :
CTCAP AND BONE SCAN FREE

BX
IDC

ER
0/8

PR
6/8

Her 2 Neu
3

SISH

KI67

BX COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T

N

M

Sonamammography

cesm 20/9/2023: bilateral moderate fibroadenosis
rt breast recent post operative changes with soft tissue lesions (residual) associated with suspicious
rt axillary LN (BIRADS 6)

PATIENT HISTORY

PT ENDED NACT 4AC+4 TAXOTERE+3 HP
ON/EX STILL RESIDUAL DISEASE ABOUT 2X2CM
BCS-ALND done: ypT1 N1/11 + POSITIVE MARGINS
Post op CESM: residual multiple breast tiny lesions
RT BCS+ALND followed by completion MRM (bec. +ve superior margin)
multi centric DCIS (high grade)
free margins , nipple , areola
LN 1/6-previos operation 1/11

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

For :

- Adjuvant Kadcylla x 12 cycles.
- PORT.
- Adjuvant Hormonal ttt (AI for 5 years , being ER 0/8 , PR 6/8 for the benefit of doubt)
- + Prophylactic zometa / 6 months.

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د. نفاحة حسن ابراهيم د. كاترينه نصيف	RADIOLOGY	RADIOLOGY د. محمد جوي
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Patient Name : الأستاذة / نفاحة حسان حسين ابراهيم	Request No. : 13007603
Patient No. : 27208312800341	Request Date : 12/08/2023
Path No. : 2616/23	Verification Date : 28/08/2023
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 50 Y	Department : Out Patient
Referreing Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, received neoadjuvant therapy
Nature of the specimen:
Right BCS and axillary clearence specimens .

Gross Examination:

Two specimens were recieved :

-Axillary clearence specimen : Fibrofatty tissue peice measured 7x6 cm , enclosing 11 nodules up to 1x1 cm
-Right wide local excision specimen measured 7x6 cm with covering skin elipse measured 5x1.5 cm

• Tumor extent

o Number: An ill defined rubbery greyish white tumor bed site .

o Size: 5x4x1.5cm

o Skin involvement: Free grossly

• Distance from margins: 0.1 cm from superior margin, 0.2 cm from inferior and deep margin (least margin)

• Frozen section:

-Lymph nodes :Free(0/3) , rest are left for parafin

-Free margins

Microscopic examination:

• Histologic type:Ductal carcinoma insitu , high grade (comedo type). No invasive component detected.

• Lymph node involvement: yes (1/11)

• Resection Margins: Superior margin involved by DCIS. Free other margins.

• Lymphovascular emboli: yes, numerous.

• Histological response to therapy: Pathologic partial response.

Verified by :

Dr.Marwa mahmoud

Printing Date : 31/08/2023 11:42:05

Page 1 of 2

Printing By : Ali mostafa

Patient Name : الأستاذة / تفاحة حسان حسين ابراهيم

Request No. : 13007603

Patient No. : 27208312800341

Request Date : 12/08/2023

Path No. : 2616/23

Verification Date : 28/08/2023

Sex : Female

Financial Category : نفقة الدولة - اورام دار السلام

Age : 50 Y

Department : Out Patient

Referring Doc :

Registered by : marwa mohamed youss

Pathology Report

Diagnosis:

Right breast cancer post NACT, wide local excision and axillary clearence specimens:

-Ductal carcinoma insitu , high grade (comedo type).

-Superior margin showed DCIS.

-Free other resection margins.

-Lymph nodes :Positive (1/11)

-Pathologic partial response

-Stage : ypTis , ypN1a.

-Comment: There is a disparity between frozen and parafin results , however final diagnosis depends on parafin results.

Prof. Gamal Abdelatty

Dr. Mienna Nabil

Verified by :

Printing Date : 22/07/2024 08:38:28

Page 2 of 2

Printing By : Mrs. Mayada Hegazy

Patient Name	: الأستاذة / تفاحة حسان حسين ابراهيم	Request No.	: 13008025
Patient No.	: 27208312800341	Request Date	: 25/10/2023
Path No.	: 3697/23	Verification Date	: 11/11/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 51 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right completion mastectomy specimen.

Gross Examination:

- * Procedure: Right completion mastectomy measured 17x13x4 cm with covering skin ellipse measured 16x9 cm showing 8cm scar.
- * Tumor extent: A greyish white surgical bed site.
- * Size: 5x2x2 cm.
- * Skin involvement: Nipple & areola were remarkable grossly.
- * Distance from margins: 1 cm away from deep margin.
- * Lymph nodes number and size: 6 L.Ns up to 1.5cm.

Microscopic Examination:

- * Histological type: Multicentric DCIS (high grade) with lobular cancerization .
- * Skin involvement: Free nipple and areola.
- * Lymph node involvement: Positive (1/6)
- * Resection Margins: Free margins.
- * Microcalcifications: Yes.

Diagnosis:

Right breast cancer post NACT, completion mastectomy specimen:
Multicentric DCIS (high grade) with lobular cancerization .
Free margins.
Free nipple and areola.
Positive lymph node 1/6.
- Stage: ypTis (m).

Signature

Verified by :





Patient Name	: الأستاذة / تفاحة حسان حسين ابراهيم	Request No.	: 13008025
Patient No.	: 27208312800341	Request Date	: 25/10/2023
Path No.	: 3697/23	Verification Date	: 11/11/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 51 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Prof. Dr. Amal Hareedy.
Dr. Heba Hassan.

Verified by :





Patient Name	: الأستاذة / نقاحه حسان حسين ابراهيم	Request No.	: 13004302
Patient No.	: 27208312800341	Request Date	: 01/04/2023
Path No.	: SR-415/23	Verification Date	: 06/04/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 50 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

CLINICAL DATA

Right breast mass

NATURE OF SPECIMEN

Revision of tru cut biopsy & IHC slides

GROSS EXAMINATION

One block (I-P-1442) + 3 slides.

MICROSCOPIC EXAMINATION

- * Histological type: Invasive duct carcinoma
 - * Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
 - * DCIS: No
 - * Angioinvasion: Could not be detected within the limits of biopsy received
- IPT :
- ER:No nuclear staining in tumor cells(0/8)
- PR:Moderate nuclear staining in 40% of tumor cells (6/8)
- Her2: Strong complete membrane staining in >10 of tumor cell , score(+3)

DIAGNOSIS

Right breast mass ; slide revision of tru cut biopsy specimen :

Invasive duct carcinoma, Grade II, exhausted material .

IPT :

ER: Negative(0/8)

PR:Positive (6/8)

Her2: Positive score(+3)

Signature

Prof. Dr. Amal Hareedy

Dr.Heba Hassan

Verified by :

Dr.Hebatallah Mostafa

PATIENT NAME: TEFFAH AH HASAAN HUSEN BRAHIM	PATIENT ID #: 2022209633
GENDER: Female	ORDER #: 2003065,2003066
AGE: 50 y	DATE OF SERVICE: 26-Nov-2022 10:22 AM

Report: - all studies

POST CONTRAST MSCT OF THE CHEST, ABDOMEN AND PELVIS

A reported case of right breast cancer;

(I) Chest findings:

- The right breast shows an enhancing soft tissue mass measuring about 32 mm in its maximum axial dimension. ✓
- Multiple enlarged ipsilateral right axillary (level I and II) lymph nodes - the largest measures 15.5x13.5 mm. ✓
- Few subcentimetric right pulmonary nodules are noted.
- Focal right upper lobar bronchiectatic changes with ? mucoid impaction.
- No pulmonary masses or cavitary lesions. ✓
- No pathologically enlarged mediastinal or hilar lymph nodes.
- Normal CT appearance of the heart and great vessels.
- No pleural or pericardial collections.

(II) Abdomen and pelvis findings:

- Enlarged liver (span = 22.5 cm) showing smooth outlines and diffuse homogeneously low parenchymal attenuation denoting fatty infiltration. No definite focal lesions or dilated biliary radicles.
- Patent homogeneously opacified portal vein and its main branches.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Distended urinary bladder with mild smooth mural thickening suggestive of cystitis for lab correlation. No gross masses, dense stones or diverticula. ✓
- Prominent left parametrial tortuous vascular structures suggestive of pelvic congestion. ✓
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No ascites.

OPINION:

- Right breast mass with enlarged ipsilateral axillary lymph nodes (level I and II)
 - Few subcentimetric right pulmonary nodules (?significance) for follow up
 - No pelvi-abdominal metastatic deposits
- 

Reported By,

Dr. Eman Zaky, MD
Dr. Ahmed Abdelmoneim, MSc

26-Nov-2022 06:54 PM

**Ain Shams University Hospitals
Radiology department**



مستشفيات جامعة عين شمس
قسم الأشعة التشخيصية

Patient Name: Tofaha Hassan Hussein Ibrahim

Patient ID:10210122142482

D/T:12/17/2022 11:36:07 AM

Tc^{99m} MDP whole body bone scan

History: 50 year old female recently diagnosed Right breast cancer, not yet initiated treatment for metastatic work up.

Technique:

Whole body anterior and posterior blood-pool and delayed 3 hours scans were taken after IV injection of 20 mCi of Tc^{99m} MDP.

Findings:

a)Bone lesions:

There are no osseous lesions suspicious for metastatic disease.

Heterogeneous tracer distribution is noted along the spine keeping with degenerative changes.

Degenerative changes of both shoulder, knee and ankle joints are noted.

b)Other findings:

- Soft tissue contours are unremarkable .
- Both kidneys are visualized.

Impression:

The surveyed skeleton is free from metastatic osseous deposits.

Much Obligated,

*R/Mariam Hossam
R/Abanoub Samuel
Ahmed Hussein,MSc
Heba Elnuggar,MD*



Patient Name: Tofaha Hassan Hussein Ibrahim

Patient ID:10210122142482

D/T:10/28/2022 9:48:19 AM

MRI of Both BREASTS:

The study was performed on a 1.5 Tesla high field machine

Technique Ax & Sag DWI, T2, STIT, 3D GRE, TIWI Pre & post IV Gad & post processing subtraction, MIP & time intensity curves were generated & revealed

CLINICAL DATA: *sensation of right breast lump, BIRADS 4C by sonomammography.*

Findings:

- The right breast shows a somehow rounded speculated heterogeneous soft tissue early enhancing mass lesion measuring 2.6 x 3 x 2.4 cm seen at 3 o clock position, 4 cm from the nipple and 2 cm from the lateral skin surface. it shows strong early post contrast enhancement with a type III rapidly rising continuous curve.
- the right axilla shows multiple right axillary lymphnodes, the largest measures 13.5 x 16 mm and shows an eccentric fatty hilum with focal cortical thickening reaching 7mm .
- the right breast shows multiple small cysts displaying fluid signal ranging in size from 2 - 7 mm and dilated retroareolar ducts reaching up to 4 mm
- Normal MRI of the left breasts parenchyma,
- both breasts display mix of fibroglandular tissue, fatty component with no detectable solid or cystic masses on the left side.
- No duct ectasia on the left side.
- No pathologically enlarged LNs seen on the left .
- Normal MRI of skin & subcutaneous tissue & nipple.
- **Conclusion:-**
- A highly suspicious Right breast soft tissue mass lesion likely malignant with a suspicious looking right axillary lymph node for histopathology correlation (BIRADS 5).
- right breast multiple small cysts and duct ectasia
- unremarkable left breast (BIRADS 1)

Much Obligated,
R.Mariam Hossam
R.Mariam Saad
R.Dalia Hesham
Nehal Fouda,MSc

Much Obligated, R/Nehal Fouda
Much Obligated,
Chris Nabil, MD

MRS: Tufaha Hassaan Hussein	Study Date: 20/09/2023
DEAR SIR	Patient ID: 967688

**BILATERAL DIGITAL COMPUTED MAMMOGRAPHY AND SONOGRAPHY
WITH CONTRAST ENHANCED MAMMOGRAPHY REVEALED:-**

Injection of 60mm of non-ionic intravenous contrast media:

A recent history of right lumpectomy one month ago, the current study revealed;

- The right breast shows thickened skin and subcutaneous tissue measuring about 4 mm, mostly post operative sequelae.
- The left breast shows normal nipple, areola, skin and subcutaneous tissue thickness.
- Both of them show condensation of glandular tissue elements, with nodularity, suggestive of bilateral moderate fibroadenosis.
- An opacity is seen at the upper quadrant of right breast associated with scattered microcalcifications.
- **Contrast enhanced mammography** confirmed this area with faint enhanced uptake is noted at right upper quadrant. Another uptake is seen at the right lymph node.
- No evidence of any contrast enhanced are detected at the left breast.
- **Sonographic examination** confirmed the mammographic findings and revealed heterogeneous area is seen at right breast mostly post operative edema with a well defined soft tissue lesion seen at the right 12 O'clock position measuring about 8 x 4 mm, other two lesions are seen at right 10 O'clock position measuring about 4 x 3 mm and 4 x 2 mm.
- Evidence of suspicious right axillary lymph node measuring about 15 mm in length with thickened its cortex.
- Left enlarged reactive axillary lymphadenopathy of inflammatory nature.

CONCLUSION:-

- **Bilateral moderate fibroadenosis.**
- **Right breast recent post operative changes with soft tissue lesions (residual), associated with suspicious right axillary lymph node (BIRADS 6).**

MUCH OBLIGED

A. PROF. DR. SAFIENAZ YOUSSEF

PROF. DR. MOHAMED ABOU EL HODA

N.I.

BI-RADS 0	Need Additional Imaging Evaluation and/or Prior Mammograms For Comparison:
BI-RADS 1	Negative:
BI-RADS 2	Benign Finding:
BI-RADS 3	Probably Benign Finding - Initial Short-Interval Follow-Up Suggested: less than a 2% risk of malignancy.
BI-RADS 4	Suspicious Abnormality - Biopsy Should Be Considered: wide range of probability of malignancy (2 - 95%).
BI-RADS 4 Category 4a	low suspicion for malignancy
BI-RADS 4 Category 4b	moderate suspicion for malignancy
BI-RADS 4 Category 4c	high suspicion for malignancy
BI-RADS 5	Highly Suggestive of Malignancy. Appropriate Action Should Be Taken: with a >95% likelihood of malignancy.
BI-RADS 6	Known Biopsy Proven Malignancy. Appropriate Action Should Be Taken

ACR Classification of breast density

- 1 = Almost entirely fatty
- 2 = Scattered areas of fibro-glandular density
- 3 = Heterogeneously dense
- 4 = Extremely dense